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File No.: EXP202204002

RESOLUTION No.: R/00140/2023

In view of the complaint filed with this Agency by A.A.A. (hereinafter, the complainant), against HM HOSPITALES 1989, S.A. (hereinafter, the respondent), for not having duly attended to their right of access.

After carrying out the procedural actions provided for in Title VIII of Organic Law 3/2018, of December 5, on the Protection of Personal Data and Guarantee of Digital Rights (hereinafter LOPDGDD), the following FACTS have been established:

FIRST: The complainant exercised the right of access to the complete copy of the medical history of their deceased sister, which has been denied.

The complainant, in light of the denial, filed a complaint with this Agency, which forwarded the complaint to the respondent for analysis and response to this Agency within one month.

The respondent replied by stating that the deceased expressed her wish that her medical history not be communicated to anyone.

In view of this response, the Agency concluded with a first resolution of dismissal, dated June 29, 2022, namely:

“...once the reasons presented by the respondent, which are in the file, have been analyzed, it is considered that the rejection of the request made by them is in accordance with the personal data protection regulations, and it is not necessary to urge the adoption of additional measures as the requested right has been attended to, therefore, in view of the actions taken, the dismissal of the complaint presented is appropriate...”

The complainant does not consider the right to have been attended to and files a motion for reconsideration dated July 18, 2022, which is resolved favorably and leads to the current resolution.

The reasons that led this Agency to uphold the appeal with a resolution date of November 23, 2022, were:

“...In relation to the statements made by the appellant, it is argued that the respondent has not proven by means of a signed document from her sister that she opposed granting access to her family to her medical history after her death, as established in Article 18.4 of Law 41/2002, of November 14, the basic regulatory law on patient autonomy and rights and obligations regarding information and clinical documentation (hereinafter, LAP).

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On the other hand, it adds the necessity they have for access to their deceased sister's medical history in order to understand the causes of her death, due to having similar pathologies and reactions to medications as their deceased sister...”

And it concludes by upholding the appeal, thus proceeding with the complaint regarding the unaddressed right to medical history.

Namely:

“...In the present case, in the allegations presented by the respondent, it has not been proven that the deceased expressly prohibited access to her medical history after her death. Without questioning at any time the veracity of the information contained in the medical history, it is not possible to deduce from the statements contained therein a prohibition of access to the history after her death. Therefore, in order to complete the information to substantiate the statements made by the respondent, it is appropriate to uphold the motion for reconsideration presented so that the processing of the complaint may continue...”

SECOND: In accordance with Article 65.4 of the LOPDGDD, which has provided a mechanism prior to the admission for processing of the complaints filed with the AEPD, consisting of forwarding them to

the Data Protection Delegates designated by the controllers or processors of the data, for the purposes provided for in Article 37 of the aforementioned regulation, or to them when they have not been designated, the complaint was forwarded to the respondent entity so that it could proceed with its analysis and respond to the complainant and this Agency within one month from which it would be admitted for processing.

The respondent presents allegations in which, in summary, it continues to deny access to the medical history for the same reason, the deceased's refusal. The arguments of the respondent are:

“...The existence or non-existence of such a prohibition may be proven in any legally accepted manner, but, for evidentiary purposes, it is preferable that it be documented in writing. In this regard, we can consider that the most effective means of knowing and making effective the patient's wish to prohibit access to their medical history by their relatives after their death is through the medical history itself, documenting it therein, as occurs in the case at hand, recorded in the list of evolutions, which chronologically records all consultations, data related to the health and health needs of the patient or user. It is important to mention that what is recorded in this documentary set is presumed to be done, which obliges anyone who does not consider it so to make an effort to gather evidentiary elements that disprove it, as there is a presumption of veracity. Regarding the evidentiary value of the content of the Medical History, it is well established by our jurisprudence to grant it a presumption of veracity of its content...”

FOURTH: The allegations presented by the respondent are forwarded to the complainant, so that, within a period of fifteen working days, they may present any allegations they deem appropriate. The complainant has not submitted any allegations.

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LEGAL GROUNDS

FIRST: The Director of the Spanish Agency for Data Protection is competent to resolve this matter, in accordance with the provisions of paragraph 2 of Article 56 in relation to paragraph 1 f) of Article 57, both of Regulation (EU) 2016/679 of the European Parliament and of the Council of April 27, 2016, on the protection of natural persons with regard to the processing of personal data and on the free movement of such data (hereinafter, GDPR); and Article 47 of the LOPDGDD.

SECOND: In accordance with the provisions of Article 55 of the GDPR, the Spanish Agency for Data Protection is competent to perform the functions assigned to it in Article 57, including the enforcement of the Regulation and promoting awareness among controllers and processors regarding their obligations, as well as handling complaints lodged by a data subject and investigating the reasons for such complaints.

Correspondingly, Article 31 of the GDPR establishes the obligation of controllers and processors to cooperate with the supervisory authority that requests it in the performance of its functions. In the event that they have appointed a data protection officer, Article 39 of the GDPR assigns this officer the function of cooperating with said authority.

In accordance with this regulation, prior to the admission of the complaint that gives rise to this procedure, the complaint was forwarded to the responsible entity for analysis, requiring it to respond to this Agency within one month and to demonstrate that it has provided the complainant with the appropriate response, in the event of exercising the rights regulated in Articles 15 to 22 of the GDPR. This admission agreement determines the initiation of the present procedure for the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of the GDPR, regulated in Article 64.1 of the LOPDGDD, which states:

“1. When the procedure refers exclusively to the lack of attention to a request for the exercise of the rights established in Articles 15 to 22 of Regulation (EU) 2016/679, it shall be initiated by an admission agreement, which shall be adopted in accordance with the provisions of the following article.

In this case, the deadline for resolving the procedure shall be six months from the date on which the admission agreement was notified to the complainant. After this period, the interested party may

consider their complaint to be upheld.”

The clarification of administrative responsibilities within the framework of a sanctioning procedure is not deemed appropriate, given that its exceptional nature implies that, whenever possible, preference should be given to alternative mechanisms supported by current regulations.

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It is the exclusive competence of this Agency to assess whether there are administrative responsibilities that should be clarified in a sanctioning procedure and, consequently, the decision regarding its initiation, with no obligation to commence a procedure upon any request made by a third party. Such a decision must be based on the existence of elements that justify the initiation of sanctioning activity, circumstances that do not occur in the present case, considering that with the present procedure, the guarantees and rights of the claimant are duly restored.

THIRD: The rights of individuals regarding the protection of personal data are regulated in Articles 15 to 22 of the GDPR and Articles 13 to 18 of the LOPDGDD. The rights of access, rectification, erasure, objection, the right to restriction of processing, and the right to data portability are contemplated. The formal aspects related to the exercise of these rights are established in Articles 12 of the GDPR and 12 of the LOPDGDD.

Furthermore, consideration is given to what is expressed in Recitals 59 and following of the GDPR. In accordance with the provisions of these regulations, the data controller must establish formulas and mechanisms to facilitate the exercise of rights by the data subject, which will be free of charge (without prejudice to the provisions of Articles 12.5 and 15.3 of the GDPR), and is obliged to respond to requests made no later than one month, unless it can demonstrate that it is unable to identify the data subject, and to express its reasons in the event that it does not intend to comply with such request. The burden of proof regarding compliance with the duty to respond to the request for the exercise of rights made by the affected party lies with the controller.

The communication addressed to the data subject regarding their request must be expressed in a concise, transparent, intelligible, and easily accessible manner, using clear and simple language. Regarding the right of access to personal data, in accordance with the provisions of Article 13 of the LOPDGDD, when the exercise of the right refers to a large amount of data, the controller may request the affected party to specify the "data or processing activities to which the request refers." The right will be considered granted if the controller provides remote access to the data, being deemed that the request has been addressed (although the data subject may request the information referred to in the aspects provided in Article 15 of the GDPR).

The exercise of this right may be considered repetitive more than once during the period of six months, unless there is a legitimate reason for it.

On the other hand, the request will be considered excessive when the affected party chooses a means other than the one offered that incurs a disproportionate cost, which must be borne by the affected party.

FOURTH: In accordance with the provisions of Article 15 of the GDPR and Article 13 of the LOPDGDD, "the data subject has the right to obtain from the data controller..."

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confirmation of whether or not personal data concerning you is being processed and, in such case, the right of access to personal data.”

Like the other rights of the data subject, the right of access is a personal right. It allows the citizen to obtain information about the processing of their data, the possibility of obtaining a copy of the personal data concerning them that is being processed, as well as information, in particular, about the purposes

of the processing, the categories of personal data concerned, the recipients or categories of recipients to whom the personal data have been or will be communicated, the expected retention period or criteria for retention, the possibility of exercising other rights, the right to lodge a complaint with the supervisory authority, the information available about the source of the data (if it has not been obtained directly from the data subject), the existence of automated decisions, including profiling, and information about transfers of personal data to a third country or to an international organization. The possibility of obtaining a copy of the personal data being processed shall not adversely affect the rights and freedoms of others; that is, the right of access shall be granted in such a way that it does not affect third-party data.

The right of access in relation to medical records is specifically regulated in Article 18 of Law 41/2002, of November 14, Basic Law Regulating Patient Autonomy and Rights and Obligations in Information and Clinical Documentation (hereinafter LAP), the literal text of which states:

“1. The patient has the right of access, subject to the reservations indicated in section 3 of this article, to the documentation of the medical record and to obtain a copy of the data contained therein. Health centers shall regulate the procedure that guarantees the observance of these rights.

2. The patient's right of access to the medical record may also be exercised by duly accredited representation.

3. The patient's right of access to the documentation of the medical record may not be exercised to the detriment of the right of third parties to the confidentiality of the data recorded therein in the therapeutic interest of the patient, nor to the detriment of the rights of the professionals involved in its preparation, who may oppose the right of access based on the confidentiality of their subjective notes.

4. Health centers and individual practitioners shall only provide access to the medical records of deceased patients to persons linked to them, for family or factual reasons, unless the deceased has expressly prohibited it and this is evidenced. In any case, access by a third party to the medical record motivated by a risk to their health shall be limited to the relevant data. Information affecting the privacy of the deceased or the subjective notes of the professionals, or that harms third parties, shall not be provided.”

In this regard, it is important to highlight Article 15 of the LPA, which outlines the minimum content of the medical record:

“1. The medical record shall include the information deemed essential for the truthful and updated knowledge of the patient's health status. Every patient or user has the right to have a record, in writing or in the most appropriate technical support, of the information obtained in all their care processes, carried out.

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for health services both in the field of primary care and specialized care.

2. The main purpose of the medical record will be to facilitate healthcare, documenting all data that, in the medical opinion, allows for a truthful and updated understanding of the health status.

The minimum content of the medical record will be as follows:

a) Documentation related to the clinical-statistical sheet.

b) Admission authorization.

c) Emergency report.

d) Anamnesis and physical examination.

e) Progress notes.

f) Medical orders.

g) Consultation sheet.

h) Reports of complementary examinations.

i) Informed consent.

j) Anesthesia report.

k) Operating room report or delivery record.

- l) Pathological anatomy report.
- m) Nursing care progress and planning.
- n) Therapeutic application of nursing.
- ñ) Vital signs chart.
- o) Clinical discharge report.

Paragraphs b), c), i), j), k), l), ñ), and o) will only be required in the completion of the medical record when it concerns hospitalization processes or as stipulated.

3. The completion of the medical record, in aspects related to direct patient care, will be the responsibility of the professionals involved.

4. The medical record will be maintained with criteria of unity and integration, at each healthcare institution at a minimum, to facilitate the best and most timely understanding by the healthcare providers of the data of a specific patient in each care process.”

Regarding the preservation of the medical record, Article 17 of the LPA, in its points 1 and 5, states that:

“1. Healthcare centers have the obligation to preserve clinical documentation under conditions that guarantee its proper maintenance and security, although not necessarily in the original format, for the proper care of the patient for the appropriate time for each case and, at a minimum, five years from the date of discharge from each care process...”

5. Healthcare professionals who carry out their activity individually are responsible for the management and custody of the care documentation they generate.”

FIFTH: In the case analyzed here, the claimant exercised the right of access to the medical record of her deceased sister, and her request was not attended to. The respondent denied access from the beginning, claiming that the deceased had expressed such a wish.

Initially, this Agency filed the complaint considering that it had been denied with justification, but the claimant submitted a request for reconsideration arguing that the respondent had not proven the deceased's refusal.

Now, we resolve this complaint after the reconsideration request was upheld, reanalyzing all circumstances and concluding that it is not lawful for the respondent to deny access to the medical record without first proving the deceased's refusal and her clear will in this regard.

In this sense, it should be noted that the assertion made by the respondent regarding the existence of an explicit refusal by the deceased for her family to access her medical record has not been accompanied by supporting evidence. As stated by the respondent in their writing, there is a need for evidence of an express prohibition that, we insist, has not been provided either to the respondent or to this Agency. This is because the statements included in the list of progress notes provided by the respondent cannot be identified as such, as they refer to the patient's refusal to inform her family of her condition during her illness, which is a right recognized in Article 5 of the LPA.

In this file, what is evaluated is the existence or not of an express prohibition by the deceased regarding access to her medical record by her family, and, we insist, this has not been proven, so access cannot be denied provided that the other required conditions are met.

Based on the above, considering that this procedure aims to ensure that the guarantees and rights of the affected parties are duly restored, and given that the right has been denied without proof, this complaint is upheld.

In view of the cited provisions and others of general application, the Director of the Spanish Agency for Data Protection RESOLVES:

FIRST: TO UPHOLD the complaint filed by A.A.A. and to instruct HM HOSPITALES 1989, S.A. with NIF A79325858, to send to the claimant, within ten working days following the notification of this resolution, a certification stating that it has addressed the right of access exercised by her or justify to the claimant that her request has been duly addressed / or deny it with justification indicating the reasons why it is not appropriate to attend to her request. The actions taken as a result of this Resolution must be communicated to this Agency within the same period. Non-compliance with this resolution could result in the commission of the infringement considered in Article 72.1.m) of the LOPDGDD, which will be sanctioned in accordance with Article 58.2 of the GDPR.

SECOND: NOTIFY this resolution to A.A.A. and HM HOSPITALES 1989, S.A.

In accordance with the provisions of Article 50 of the LOPDGDD, this Resolution will be made public once it has been notified to the interested parties.

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Against this resolution, which concludes the administrative procedure in accordance with Article 48.6 of the LOPDGDD, and in accordance with the provisions of Article 123 of the LPACAP, the interested parties may optionally file a motion for reconsideration before the Director of the Spanish Agency for Data Protection within one month from the day following the notification of this resolution or directly file a contentious-administrative appeal before the Contentious-Administrative Chamber of the National Court, in accordance with the provisions of Article 25 and Section 5 of the fourth additional provision of Law 29/1998, of July 13, regulating the Contentious-Administrative Jurisdiction, within two months from the day following the notification of this act, as provided in Article 46.1 of the aforementioned Law.
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